



The State and Civil Society: Meeting health needs, reaching equity

SHOBHA RAGHURAM
AND MANASHI RAY¹

ABSTRACT *Shobha Raghuram and Manashi Ray examine the health standards for the poor in India within the broader context of human development concerns during the last decade, with special emphasis on the role of the state and its commitment towards achieving 'health for all'. They discuss the response of civil society in issues of public health and in monitoring the accountability of the state, underscoring the need for reforms in the state services.*

Globalization and health

While globalization has been thought of as positively contributing to setting standards globally, the divides in quality of life have become even more stark and worrisome. Health has been a sector of silent neglect in many countries receiving scant attention when it comes to financing all related public health needs. The severe local and global imbalances in the enjoyment of health call for radical measures that will bring greater equilibrium in people's access to health. The minimizing of the role of state and the increasing reliance on market mechanisms to address welfare needs during the reform period has not solved the issue of millions of people globally living with unmet needs. The discussion on globalization, equity and health should not be laboured under the same rubric of growth and liberalization. The state may indeed have performed too little in the areas of basic needs. What is required is the maximization of the state in providing sufficient conditions for basic needs provisions. Sen and Dreze (1996), when writing about intrinsic value and human capabilities, drew the attention of policy makers to the excessive concentration on issues of liberalization and market-led changes which have detracted from the fact that the social sector areas have suffered from too little role of government and not the other way around. It is clear that the grueling data of existing poverty in the country has completely influenced the profile of ill health. The political economy of health will in the coming years be a critical area of work as poverty, health and development closely interconnect, thus influencing the overall arena of social equality.

Role of state and equity: the India case

The last 51 years of development history for India have shown the limits to policy, and the greatest need for consolidated social and political action taking place at the most micro-levels and at the same time the need to evaluate national governments which, in the larger sphere of state role, advocate policies which undermine human development. Historically, after independence, the government, in response to the large numbers of poor in unequal health circumstances, made many committed pronouncements pertaining to improving the lot of the vulnerable in the official planning processes. The 1978 'Health for All' Declaration received a full-fledged commitment from the Indian government and this was important in order to propose a broad and consistent philosophy towards a strategy secured in the primary health care PHC approach.

Today, an examination of the status of health indicates that there is surprising unevenness across borders. In addition to the rural/urban divide, the caste, class and gender disparities in health persist, giving rise to a situation in which different pockets of population within the same country portray differing health status. The availability of access to social infrastructure is uneven between social groups, economic classes and geographical regions. The scheduled classes and tribes, in particular, and the poor, in general, are being adversely affected. Health problems associated with underdevelopment (among the poor and the disadvantaged) and the extent of transitions which are creating a 'polarization' of health between different groups have been the grounds for public action. For policy-makers designing appropriate policy but also measuring the benefits of competing health investments on different groups of population is critical.

Social development and health during the reform years

There has been a great deal of discussion regarding the negative effects of the reform measures on the social sectors, especially in countries like India with high figures of poverty and its attendant forms of destitution.

The SAP initiative adjustment programmes in mid-1991 led to expenditure compression in Union Government finances in the first 2 years. The situation improved in subsequent years with an increase in the allocation to rural development, education and health. The cuts were severe in the first two years of the reforms, followed by some restoration in the next four years. The greatest casualty was health care. As opposed to this, the almost exclusively centrally funded family planning programme fared much better even in the worst resource crunch years.

The ideological underpinnings associated with the reforms package legitimizes the state's withdrawal from infrastructural areas, including infrastructure in social sector. The projected inability of the state to finance health care has been further used as a lever for the privatization of health care.

During the decade of the 1990s, with the international and bilateral funding of some health programmes there was a gross distortion of priorities in disease control and health development. The 1992 budget had seen one quarter of the planned Central Health Budget being allocated for HIV/AIDS, while the budget for malaria control had been decreased by 43 percent. Major diseases like tuberculosis and diarrhoea which cause high morbidity and mortality were neglected. It is important to note that the greatest killers in the post-reform period continue to be cholera, TB, malaria, and kala azar (Voluntary Health Association in India, 1997). In the case of malaria, the incidence figures have been at roughly 2 million cases every year, affecting mostly tribal populations. On the whole, after the initiation of economic reforms in the 1990s, the sustainability, health priorities and the future of public health programmes have been greatly influenced by external assistance and have not been pro-poor or pro-people. It is difficult to assess the extent to which the reforms have had an adverse affect on health in the 1990s. However, it can be postulated that because of considerable decline in the public expenditure in the first few years of the reforms, there might have been adverse impact on the overall human development indicators.

Health security and the poor: a matter of responsibility?

The state as the primary and critical actor in ensuring equity urgently requires the consideration of an overall plan which can strategically strengthen the sector of health in the overall plans of development and poverty eradication. The financing of poverty alleviation programmes as well as social sector services requires a comprehensive effort both on the field and in the budget exercises. Health policy will increasingly need to be designed keeping in mind both national and local agendas.

Market considerations have often flawed an overall social justice agenda and the wider canvas of human security is the basis for the serious management of public health. The primary objective of the National Health Policy (NHP) which was adopted in 1983 was to attain the creation of an infrastructure for primary health care. Other major priority areas were: close coordination of health related services and activities like drinking water supply, sanitation and nutrition; the active involvement and participation of voluntary organizations and the provision of essential drugs and vaccines.

The 1980s witnessed a massive infrastructure expansion and the creation of programmes for providing family health care facilities. However, in spite of the vast expansion in infrastructure it failed to work due to poor facilities, inadequate supplies, insufficient effective person hours, and the lack of proper monitoring and evaluating mechanisms. A bureaucratic approach, insensitive to recognizing and involving people's organization in the implementation of the family welfare programmes in the country forced the continuation of the use of private health services.

The other issue has been the effectiveness of expenditure incurred in terms of human development outcomes, and the relative importance of economic growth and public provisioning in capability enhancement of states (Chelliah and Surdarsan, 1999). Thus, there are severe imbalances in India between public and private health care, within public health care between preventive and curative services, between primary, secondary and tertiary health care services, and between salary expenses and other recurrent expenditures, as well

as between more vulnerable, poor areas and the privileged urban centres.

Regional diversities and contrasts in health status

Progress over the decades has been uneven and remains confined to more advanced states and, even within them, to selected regions only. The health status of a large part of the Hindi speaking areas of north India, as well as the states of Orissa and Assam, remains adamantly vulnerable.

Further, the more remote, sparsely populated and resource-poor an area is, the greater are the chances of its neglect, in terms of the availability and effectiveness of health services. The flood-prone areas of Assam and Bihar and states lying along the Bay of Bengal suffer from the endemic presence of vector-borne diseases and frequent damage to infrastructure. The people in these areas – be they tribals, scheduled castes, displaced or migrant populations in search of work – remain ignored and are compelled to accept a lower quality of life. Preventive and public health care measures and primary health care services do not reach them adequately. Therefore, the political economy impinging on health policy development and its implementation has given rise to the conspicuous disparity in the health outcomes. As a result, there have been neglected areas, forgotten populations and overlooked issues. Each of these distortions is damaging enough, but when all three exist the situation is fairly acute.

Reform of the State and the role of civil society: a case for cooperative public action

Issues taken up by grassroots voluntary organizations include food security, political participation, health, urban poverty, migration, and land alienation. Governments have many mechanisms at their disposal for replicating good models that might emerge from the extensive micro-level experience that non-governmental organizations have of given communities, which help them to perceive genuine interests and to build organic links on the field. In the area of health, which

involves a spectrum of collaborative efforts, politically voluntary organizations need their autonomy from governments, as they instrumentalize different social and political interests. The mutual accountability that is being demanded of government-voluntary organizations relations requires institutional integrity (e.g. transparency), particularly to the communities involved. How they may be asked to integrate their efforts with those of the state, which is itself in the process of integration with global interests on terms clearly not set by the people themselves, is a thorny and painful issue. Social justice, accountability, high standards of morality and equality are issues that cannot be realized historically without majority consensus. For voluntary organizations or social movements the challenge lies in enabling basic needs and democracy mainstream concerns, cutting across social strata and class interests. Local bodies offer today the only hope in benevolent, decentralized state presence, where the centre may hold and the peripheries may continue to define the political spaces.

Co-operation in health strategies

The sharp gender, poverty, caste lines and the scarcity of resources make the need for extensive field co-operation an urgent matter. Communication is quite central to the effective success of programmes in public health. Health workers are the central agencies of the policy framework being materialized on the field and often this is a neglected fact. Barefoot health workers, centrally involved in communication, are often shoddily treated in their working conditions, creating a tremendous demoralization in programmes. Particularly in preventive work, information dissemination and communication, and in pressurizing the inflexible medical bureaucracies to extend support in implementation, NGOs today are making their presence felt. The work of NGOs is preventive, proactive, promotive and emancipatory. They have shown the strength of being embedded in community survival issues. Their ideas on information, education and communication (IEC) cover a large number of problems faced by the poor and by women.

Conclusion

We live in times where the poverty of others is tolerated and where women's lives, especially when they are poor, are a devalued social goal. Clean water and sanitation, universal primary education, especially for girls, more effective distribution of food and nutrition, the alleviation of poverty and greater public awareness and participation in health preservation and care are all critical components of genuine health and development strategies. Many challenges face public health policy makers. Can universal care be afforded and managed by the public sector alone? How can appropriate fiscal, human resources, and social systems be developed to achieve this goal? Could decentralization of health policy help in this process? How best can the public and private sectors be made to interact to achieve better health outcomes?

The time has come for governments to pay attention to all models of health care and campaigns which worked. This needs at regional levels both a convergence of social principles and a divergence, which may be necessary for ensuring local acceptability and success – an approach which is united on being empowering to all users, sustainable and sensitive to unequal relations of power such as class, gender and race. An action plan needs to be common to both government and private health care financing matching programmes, strategies, financial outlays and exercises for dovetailing. All frameworks need to be moving in the same direction with predetermination of strategic entry points. In tribal areas the most successful health strategies included livelihood issues, food security, literacy to enable the tribals to access their rights, e.g. food provisioning etc. Approximately they fall in the rubric of gender, class and may be judged on the basis of effectivity and accountability. Strategic interests and an overall framework cannot be oppositional in character, as is often the case. The state services with their vertical structure need to be interfaced with building horizontal linkages and working with disprivileged groups. Participation is a much used word but it is not something that can be engineered by funding and policy determinations alone. Few donors even seriously consider

Development 42(4): Negotiating New Health Systems

financing people-linking efforts, jointly evolving approaches which involve all stakeholders.

Effecting changes which are visible is a challenge. If we wish to take this challenge there is little time left. The field conditions must recall to us the unfinished agendas of existence, well-being, and the central ethical tenets of caring for others. Health must not be perceived as an economic good

and a private privilege alone. All social forces involved in health in the next millennium may wish to pledge and act on a principle which functions by inclusion and not by exclusion. Health is a social claim, and unless we pinpoint the gaps and find solutions for the structural imbalances we cannot create those dimensions which will carry forward the necessary transformatory processes.

Note

- 1 We thank Hemalatha D. at Hivos for all the valuable assistance in the preparation of the longer version of this article. For a more detailed version of this article see 'Globalisation and Equity in Health', *Technical Report Series* 1.8. Bangalore: Hivos. Forthcoming.

References

- Chelliah, R.J. and R. Sudarshan (eds) (1999) *Income-Poverty and Beyond-Human Development*. New Delhi: Social Science Press.
- Sen, A. and J. Dreze (1996) *Indian Development: Selected Regional Perspectives*. New Delhi: Oxford University Press.
- Voluntary Health Association in

India (1997) *Report of the Independent Commission on Health in India*. New Delhi.